MEDCONTRACTINTEL

VOL. I · FOUNDATIONS

Volume I of the MedContractIntel Compensation Series

The wRVU Playbook

Understand Your Pay. Know Your Worth. Negotiate What You've Earned. A Complete Guide for Internal Medicine Physicians.

Data. Leverage. Fair Pay.

MedContractIntel Compensation Series · Volume I
Internal Medicine · Outpatient + Hospitalist + Nocturnist

Internal Medicine Edition · 2026

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How to use this book

wRVU Playbook is structured to be read in order or used as a reference. Each chapter stands on its own, but the chapters build on each other. Here's where to look for what you need most.

Read in order if you're new to wRVU compensation.

Part I (Chapters 1–3) builds the foundation: how wRVUs work, how your pay is calculated, and how to compute your effective rate per wRVU. Most physicians have never seen this in one place.

Use Part II as a reference at contract time.

Chapters 4–7 cover benchmarking, the employer's revenue model, payer-mix economics, and how to read a compensation schedule line by line. Pull this part out before any review with HR.

Skip to Part III if your contract is the problem.

Chapters 8–10 cover the three IM-specific traps: panel size, structurally unreachable quality bonuses, and the salary-to-production switch at renewal. Diagnose your contract before you negotiate it.

Use the appendices as quick-reference cards.

Appendix A is the wRVU-and-reimbursement table for every IM/hospitalist code. Appendix B is the benchmark reference card. Appendix C is the 15-flag red-flag summary. Appendix D is non-compete law by state.

Pair this book with the Negotiation Script Pack.

This book teaches you the math. The Script Pack (Volume II) gives you the words. Use them together.

Scan icons

The body uses four small marks to help you skim. Each labels what kind of paragraph it leads.

■	**DATA**	A key benchmark or specific dollar figure you can cite
▲	**WATCH**	A common employer trap or below-market structural pattern
★	**WIN**	A move that compounds in your favor over the contract life
■	**DO**	A concrete action — pull this number, request this clause, do this thing

Cross-references to other titles in the MedContractIntel Compensation Series (the Internal Medicine Physician Contract Red Flag Checklist, the Negotiation Script Pack, the wRVU Playbook, and the Hospitalist Shift Economics Analyzer) appear in the body where relevant. Each title is independent but designed to complement the others.

Foreword

Pay, Power, and the Page You're About to Read

Internal Medicine is the largest physician specialty in the United States. It is also one of the most structurally disadvantaged in contract negotiations. The reason is not that IM physicians are worse negotiators than surgeons or cardiologists — it is that the information environment in which IM contracts are negotiated is tilted against the physician in specific, identifiable ways.

This book closes that gap. It does not assume you know what a wRVU is, and it does not assume you do not. It walks through the mechanics of how your pay is calculated, the specific ways IM contracts are structured to disadvantage the physician, the benchmark data that applies to your specialty, and the specific conversations that move numbers. By the end, you will have the same information the compensation analyst on the other side of the table has — which is all you need to negotiate on equal footing.

$52,000

Five-Year Cost of a $2/wRVU Shortfall

At 5,200 annual wRVUs

Chapter 1

The Information Gap

Internal Medicine is the largest physician specialty in the United States. It is also one of the most structurally disadvantaged in contract negotiations. The reason is not that IM physicians are worse negotiators than surgeons or cardiologists — it is that the information environment in which IM contracts are negotiated is tilted against the physician in specific, identifiable ways.

Start with what the employer has. When a hospital compensation analyst prepares an offer for a hospitalist position, that analyst has multiple compensation surveys, regional breakouts, a fair-market-value opinion from an outside valuation firm that has reviewed the compensation structure, and internal data on what other physicians in the organization earn. That analyst knows the 25th, 50th, 75th, and 90th percentile figures for every specialty, stratified by region, practice setting, and experience level. They run this process forty to fifty times a year.

Now consider what most IM physicians bring to the same conversation. A general sense that "the market has gotten better." A number they heard from a colleague. A salary from a single recruiter email that may or may not reflect the actual local market. Possibly a vague recollection of what residency attendings earned. That is not a data position. That is a starting point for being told a number and accepting it.

This asymmetry is worse for IM than for most other specialties. A cardiologist or an orthopedic surgeon negotiates a contract with a small number of highly visible terms — typically base, production multiplier, and call compensation. An Internal Medicine contract has more moving parts. A hospitalist contract includes base, shift rate, per-shift or per-encounter production, admissions incentives, nocturnist differential, weekend differential, call structure, census caps, admitter-rounder role splits, CME, tail coverage, non-compete terms, and often a quality or patient experience bonus on top of all of that. An outpatient IM contract adds panel size, panel attribution, capitation risk arrangements, HEDIS bonus structures, Medicare Advantage risk-sharing, and RAF coding incentives. Each of those terms is a place where a suboptimal number compounds over the life of the contract.

The consequences of this information gap are not theoretical. They are cumulative. A $2 per wRVU shortfall on a contract generating 5,200 annual wRVUs is $10,400 per year. Over a typical five-year employment period, that is $52,000 in lost compensation. A $15,000 shortfall on base compensation over the same period is $75,000. A misallocated nocturnist differential can easily exceed $100,000 over five years. None of these are hypothetical. They are the routine outcomes of contracts negotiated without benchmark data.

“You are not a cost center subsidized by the hospital. You are a revenue-generating clinician whose compensation should be calibrated to the market rate for your clinical services.”

The information gap also shapes the dynamics of negotiation itself. When you walk into a conversation without data, the compensation analyst controls the frame. The analyst decides which numbers are "market." The analyst decides which data source applies. The analyst can cite internal equity concerns, budget constraints, or regional adjustments without being challenged. When you walk in with current published 2025 data, the frame changes. You are not asking for favors. You are reconciling your compensation against the same benchmarks the organization uses internally. That shift of frame is often more valuable than any specific number you bring.

This guide focuses on Internal Medicine — primarily outpatient IM and hospitalists, including nocturnists. The fundamentals are applicable to other primary care specialties, but the specific benchmarks, CPT codes, and contract mechanisms are IM-specific.

Chapter 2

wRVU Fundamentals

A Relative Value Unit is a number that Medicare uses to describe how much work, overhead, and liability exposure is associated with a specific medical service. Every CPT code has three RVU components: work RVUs (wRVU), practice expense RVUs (peRVU), and malpractice RVUs (mpRVU). The three add up to a total RVU for the service. Medicare multiplies the total RVU by a dollar-denominated conversion factor (CF) to calculate the Medicare reimbursement for that service. That is the full mechanic.

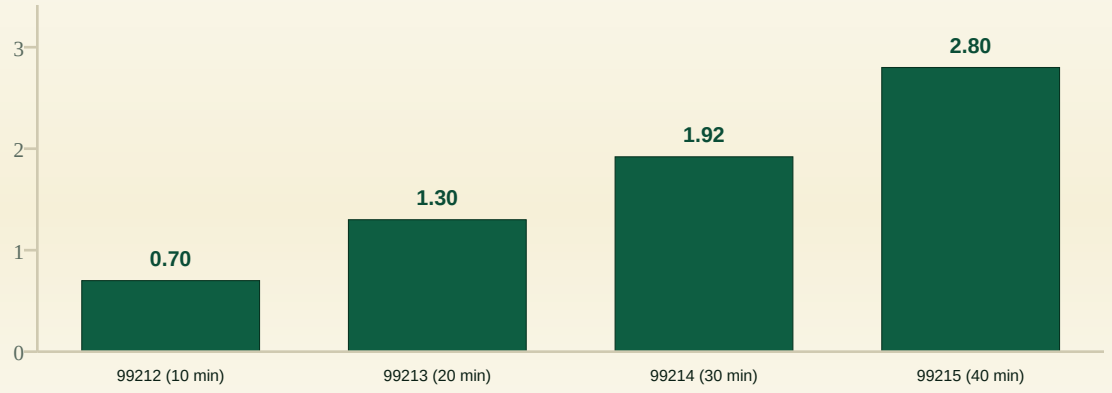
You only need to understand the first component in detail. Work RVUs are the portion of the total that reflects physician time, mental effort, technical skill, physical effort, and stress. They are the component your employer uses in your compensation contract. Practice expense RVUs cover the overhead of running the office — staff, rent, supplies, equipment — and are captured by the practice, not the physician. When your contract says "compensation is $52 per wRVU," the compensation analyst is referring only to the work RVU component. Practice expense and malpractice RVUs are not in the calculation.

This matters because wRVUs are roughly half of total RVUs for a typical office-based service. When your practice collects a Medicare payment, you see about half of the underlying RVU structure flow through to your compensation. The rest goes to the practice to cover overhead and malpractice. Private insurers pay based on the same RVU structure but at different conversion factors — usually 110–140% of Medicare rates.

The 2026 wRVU values for Internal Medicine

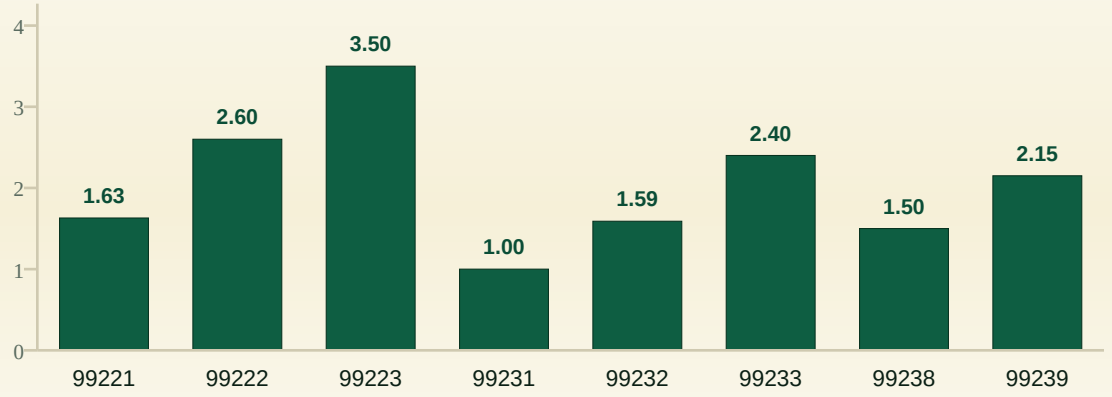
The values below are CMS CY2026 Final Rule, January 2026 release. The CY2026 rule applied a –2.5% efficiency adjustment to the work RVUs of non-time-based services; time-based E/M codes were exempted from that adjustment.

Outpatient Established Patient Visits — 2026 wRVU Values



Source: 2025 IM_DATA · CMS PFS 2026

Hospital Inpatient E/M — 2026 wRVU Values



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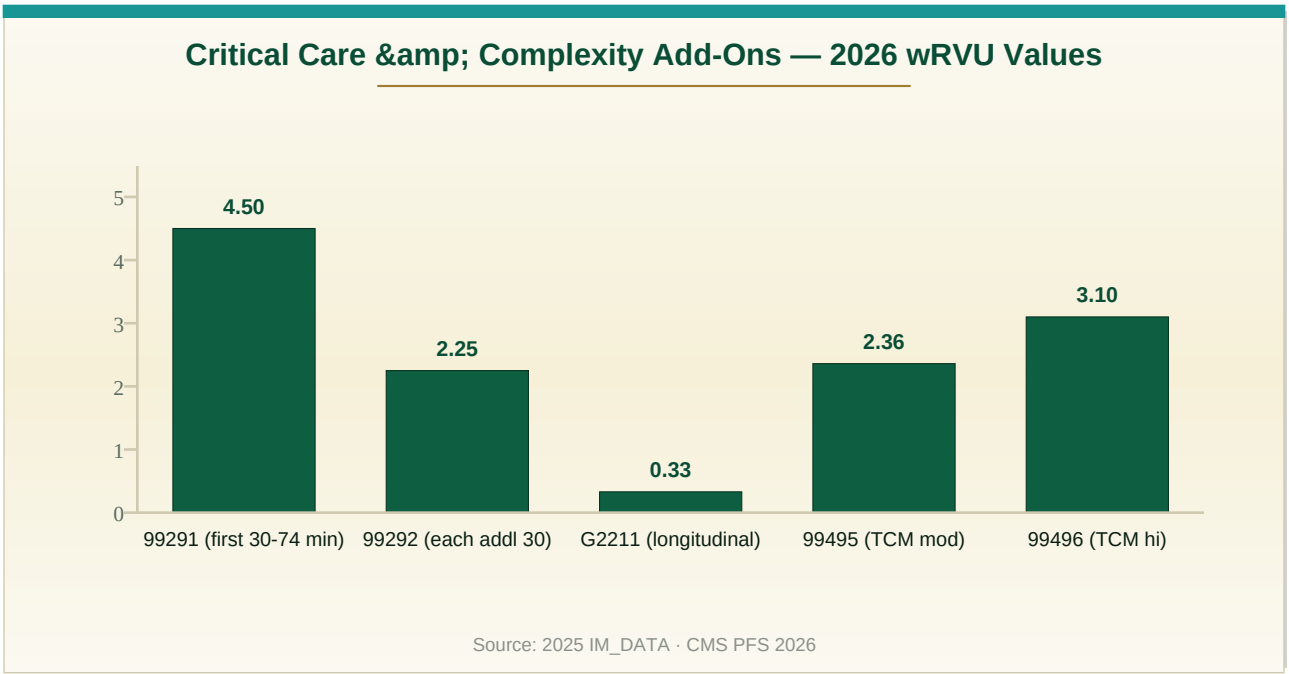


■ Several things to notice. First, the gap between a 99213 (1.30 wRVU) and a 99214 (1.92 wRVU) is 48% — nearly half a wRVU per visit. For a physician seeing 20 patients per day, the difference between coding the entire day as 99213 versus 99214 is about 12 wRVUs per day, or roughly 2,600 wRVUs per year. At $52 per wRVU, that is $135,000 in annual compensation. Accurate coding is not a minor housekeeping issue. **It is compensation.**

Second, the hospitalist codes show why admission-weighted days are more productive than discharge-weighted days. A 99223 admission is 3.50 wRVUs. A 99238 discharge is 1.50 wRVUs. If your shift structure loads admissions on Day 1 of a block and discharges on Day 7, your per-shift wRVU production is not constant across the week — it is highest at the beginning and lowest at the end.

★ Third, G2211 — the longitudinal complexity add-on — is 0.33 wRVU. This is a small number that adds up over a full panel. If you bill G2211 on 60% of your Medicare visits and you see 4,000 Medicare visits per year, that is 0.33 × 2,400 = 792 additional wRVUs per year. At $52 per wRVU, **G2211 alone is roughly $41,000 in annual compensation.** Most IM physicians either do not bill it or bill it inconsistently. This is found money.

Chapter 3

The Compensation Equation

Your pay is calculated by one of four structures, or a hybrid of them. Understanding which structure you are in is the first step in understanding what your contract actually says and where the risk lives.

Structure 1: Straight salary

You earn a fixed annual base regardless of production. This is most common in first-year contracts, academic settings, and certain community health center arrangements. The math is simple. The risk lives elsewhere — usually in the renewal, where a straight-salary contract converts to production, often without a corresponding rate increase.

Structure 2: Straight production (wRVU-based)

Your compensation is a dollar-per-wRVU multiplier times your actual production. No floor, no ceiling. This is common in private physician-owned groups and in some hospitalist contracts with established physicians. The math is: (annual wRVUs) × (multiplier). At 5,200 wRVUs × $52 = $270,400. The risk is that a production shortfall is an income shortfall with no floor underneath it.

Structure 3: Base plus production above threshold

This is the dominant structure in hospital-employed IM and hospitalist contracts. You earn a guaranteed base (often $240,000–$290,000 for a hospitalist or $220,000–$270,000 for outpatient IM), and a per-wRVU bonus above a production threshold. Example: base of $270,000 with a $52/wRVU bonus on every wRVU produced above 4,800. If you produce 5,200 wRVUs, you earn $270,000 + (400 × $52) = $290,800. The threshold is the critical term — set too high, the bonus is structurally unreachable.

Structure 4: Advance draw plus reconciliation

▲ Common in production-heavy hospitalist arrangements. You receive a biweekly or monthly advance calculated as if you were on track for a target annual wRVU production. At year-end, your actual production is reconciled against the advance. If you overproduced, you receive an additional payment. If you underproduced, the shortfall is a clawback. This structure is the one most likely to produce unexpected year-end bills to the physician and should be approached with particular care.

The single equation you should know

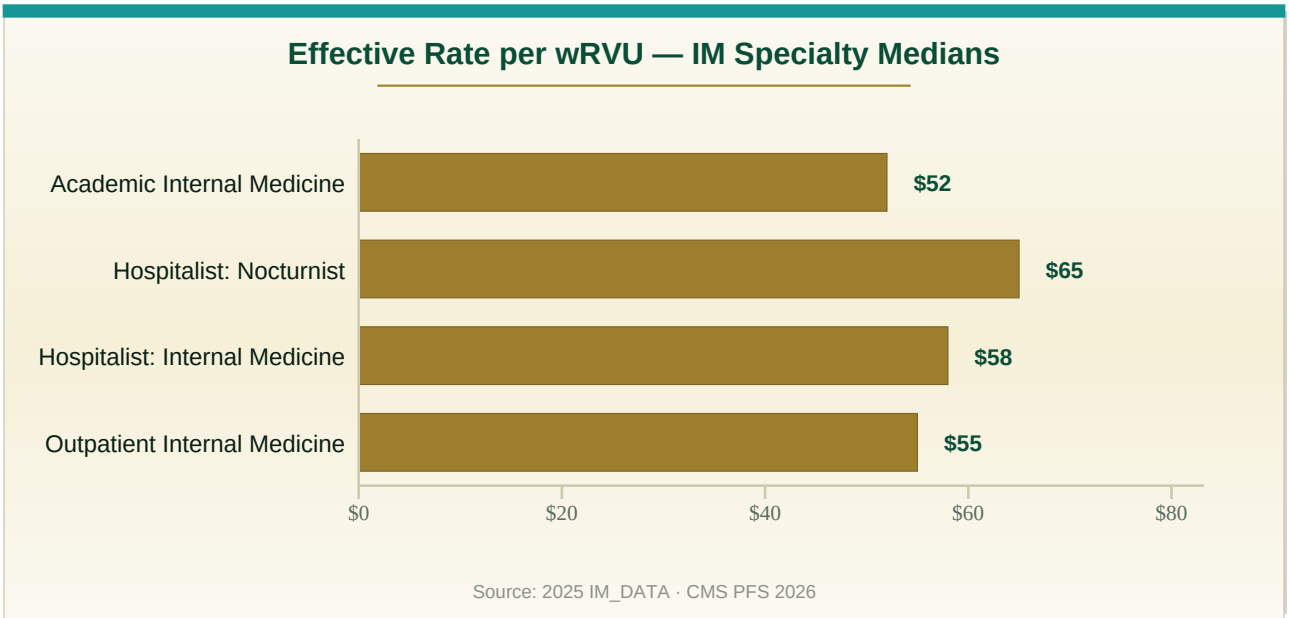
Your effective rate per wRVU is the single most useful number for benchmarking. Take your total annual cash compensation, divide by your annual wRVU production. If your effective rate is $55/wRVU and your specialty median is closer to $58/wRVU, you are below market.

Total Comp ÷ wRVUs

Your Effective Rate per wRVU

The single most useful benchmarking number

For Internal Medicine specifically, current effective rate benchmarks (synthesis of published 2025 surveys):



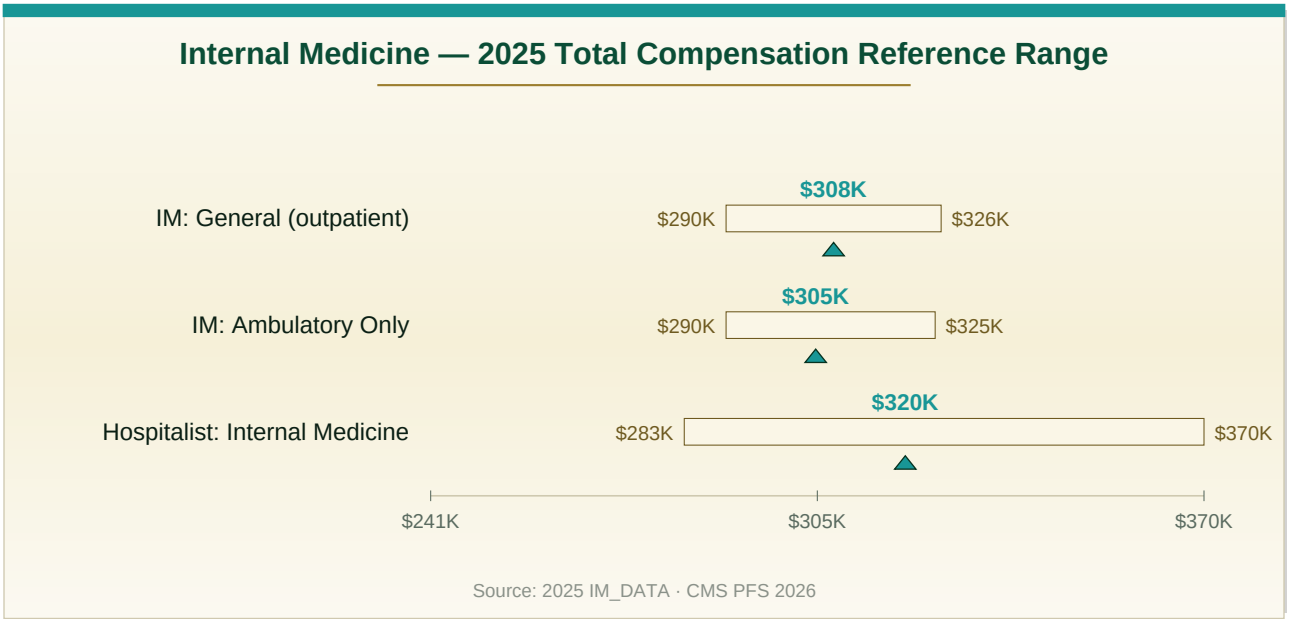
▲ Two income shortfalls show up repeatedly in IM contracts that should be on your radar. The first is the production threshold that is set at or above the 75th percentile. If the bonus kicks in only above 6,800 wRVUs (roughly the 75th percentile for Internal Medicine: General), most physicians will never touch the bonus. The second is the compensation-to-production ratio mismatch: a physician being paid at the 35th percentile of total compensation while producing at the 55th percentile.

Chapter 4

Benchmarking Your Contract

Benchmarking is the foundation of every serious contract negotiation. Without benchmarks, "I think I'm underpaid" is a feeling. With benchmarks, it is a calculation. Every hospital compensation committee, every fair-market-value opinion firm, and every physician employer of scale uses published compensation surveys to set pay. You should use the same benchmarks.

The published 2025 reference range for Internal Medicine



To benchmark your own contract against this data, you need three numbers:

1. Your total cash compensation (base plus productivity plus bonuses, fully loaded).
2. Your specialty category — choose the one that best matches your practice.
3. Your production percentile — comparing your annual wRVU production to published production benchmarks for your specialty.

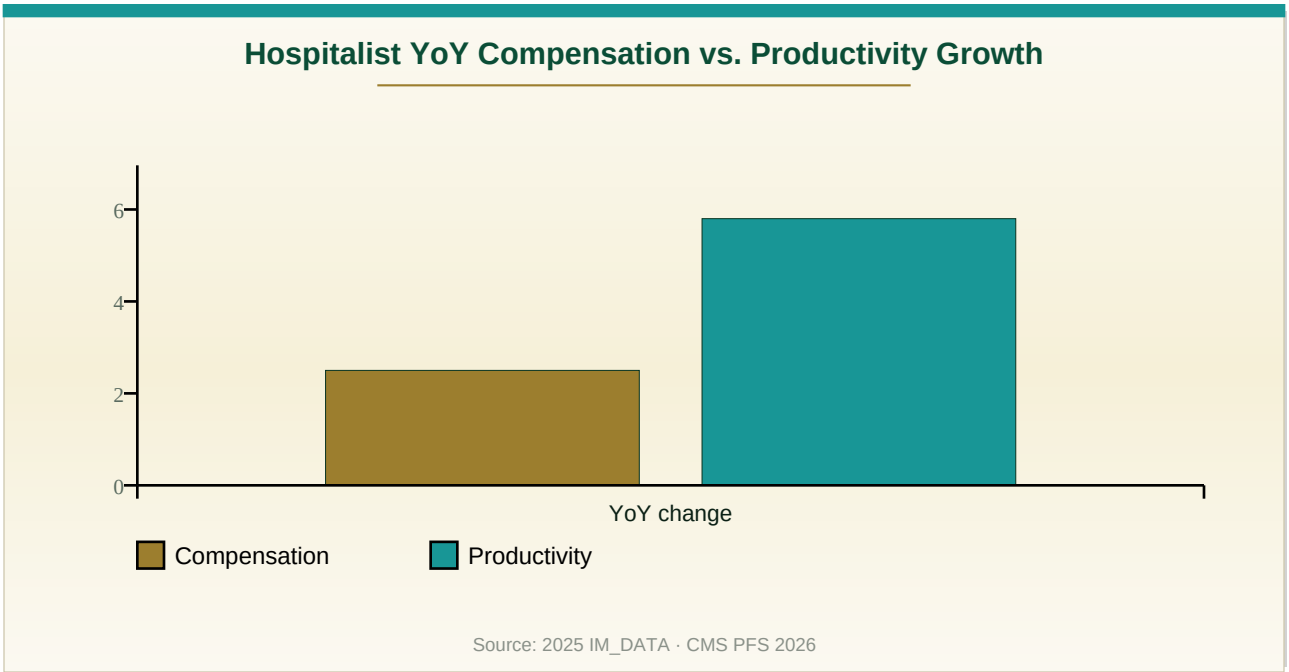
Production benchmarks and compensation benchmarks should tell the same story. If you are producing at the 60th percentile of wRVUs for Internal Medicine: General, your total compensation should be at approximately the 60th percentile. If your compensation is at the 35th percentile while your production is at the 60th percentile, there is a gap. **That gap is the quantitative basis for the negotiation.**

For nocturnists

Nocturnist compensation is most reliably benchmarked by differential rather than absolute dollars. Published surveys place the typical differential at 15–25% above base hospitalist compensation.

Productivity-vs-compensation YoY data

- Recent industry data shows hospitalist compensation grew approximately 2.5% year-over-year while productivity grew 5.8%, producing a 1.0% decrease in the compensation-per-wRVU ratio.



In a rate negotiation, this is useful — it rebuts any employer claim that "compensation rates are outpacing productivity."

“Cite the published reference range, identify where your current compensation falls, and quantify the gap. That sentence is worth more than an hour of general complaint. It converts the conversation from feelings to math.”

How to use these benchmarks in conversation: cite the published reference range, identify where your current compensation falls, and quantify the gap. *"Published 2025 surveys place the median for Hospitalist: Internal Medicine around $320,000. My current total compensation is $[X]. That places me below median by approximately $[Y]."* That sentence is worth more than an hour of general complaint. It converts the conversation from feelings to math.

Chapter 5

The Employer Model

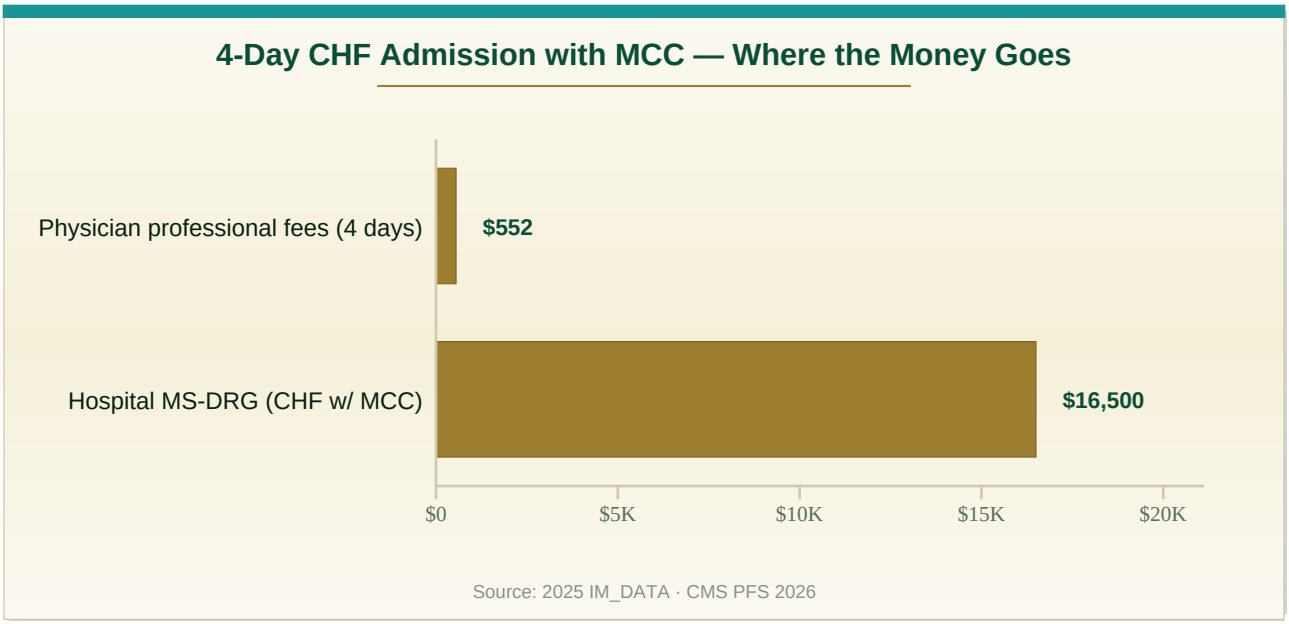
To negotiate effectively, you need to understand what your employer is making on your work. This is not about developing adversarial thinking. It is about understanding the economics of the relationship so that you can have factual conversations rather than emotional ones.

Every physician encounter generates two streams of revenue for the employer. The first is the professional fee — the payment for the physician's clinical service, billed through the physician's NPI on CMS-1500 forms. The second is either the facility fee (for inpatient encounters) or the practice's collections on overhead (for outpatient encounters). For a hospitalist, the facility fee is by far the larger stream.

A worked example: 99223 admission

When you admit a patient to the hospital and bill a 99223 (initial inpatient, high complexity — 3.50 wRVU), Medicare pays the professional fee based on the total RVU times the conversion factor. At the 2026 non-QP conversion factor of $33.40, a 99223 reimburses approximately **$156** in professional fees. Your compensation, if you are on a $55/wRVU model, would show 3.50 wRVUs × $55 = $192.50 of credit toward your production.

But the hospital's revenue on that admission is not $156. It is substantially larger. The hospital bills separately for the facility component through the MS-DRG (Medicare Severity Diagnosis Related Group) system. MS-DRG payments for medical admissions typically range from approximately $8,000 to $25,000+.



This is not a scandal — hospitals have substantial overhead, and MS-DRG payments reflect that overhead. But it is a factual reality that shapes the negotiation. When a hospitalist argues for compensation adjustment, the appropriate frame is not "the hospital should pay me more out of goodwill." The appropriate frame is **"the admission I generate produces substantial facility revenue in addition to the professional fee, and the compensation for my clinical service should reflect the market rate for that clinical service."**

Outpatient IM economics

For outpatient IM, the economics are different but the principle is the same. When you see a Medicare patient for a 99214 visit, the professional fee is approximately $136 at the 2026 non-QP rate before geographic adjustment. The practice's collections on that visit fund the practice expense component (the peRVU), which covers rent, staff, supplies, and overhead attributable to that visit. In a properly capitalized outpatient practice, the physician compensation comes from the work RVU component, and the practice's overhead is covered by the practice expense component.

The strategic takeaway: knowing that the economics of your work include more than what appears on your paystub lets you have the correct conversation. You are not a cost center subsidized by the hospital. You are a revenue-generating clinician whose compensation should be calibrated to the market rate for your clinical services.

Chapter 6

How Reimbursement Works for IM

Your compensation contract is denominated in wRVUs, but your employer's revenue is denominated in collections. Understanding the payer mix and the reimbursement rates by payer is essential for reading your contract accurately.

Medicare fee-for-service

The baseline reference point. Medicare pays the total RVU times the conversion factor. For calendar year 2026, CMS finalized two conversion factors: **$33.5675** for physicians qualifying as Alternative Payment Model (APM) participants, and **$33.4009** for all other physicians (non-QP). For most physicians, the $33.40 non-QP rate applies.

Medicare Advantage

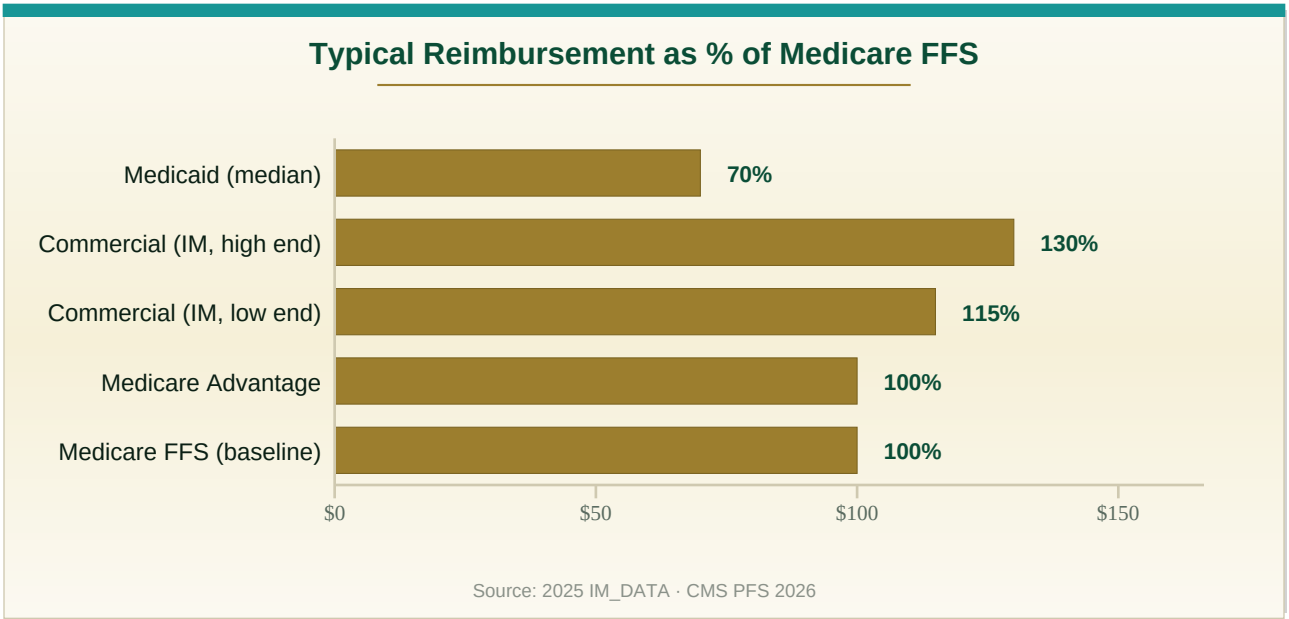
MA plans typically reimburse at 95–105% of traditional Medicare rates for most services. More significantly, MA plans introduce prior authorization, utilization management, and attribution-based risk arrangements that can meaningfully affect physician productivity and payment timing.

Commercial insurance

Commercial payers typically reimburse at 110–140% of Medicare rates, though the range is wide. Internal Medicine commercial rates are generally at the lower end of the range (often 110–125% of Medicare) because IM practices have lower negotiating leverage than specialty or procedural practices.

Medicaid

Medicaid fee-for-service typically reimburses at 60–80% of Medicare rates, varying by state.



Risk-based arrangements

A growing share of IM practice involves risk-based payment arrangements — global capitation, per-member-per-month (PMPM) payments with shared savings or full risk, and ACO Reach contracts. These arrangements do not pay per encounter; they pay per attributed patient.

How this matters for your contract

Contracts that reference "net collections" or "collection-based" compensation introduce payer-mix risk onto the physician. If your practice's Medicaid share increases, collections drop, and your compensation drops correspondingly — even if your clinical work and wRVU production are unchanged. **wRVU-based compensation insulates the physician from payer-mix risk.** This is one reason wRVU-based compensation is more favorable to the physician than collection-based compensation for most IM roles.

Chapter 7

Reading Your Compensation Schedule

Every physician employment contract includes a compensation schedule — usually an exhibit or schedule appended to the main agreement. This is where the actual numbers live. The main contract body describes the structure in general terms; the compensation schedule specifies the dollar figures. Most physicians read the main contract carefully and skim the compensation schedule. **This is the wrong priority.**

Base compensation

Confirm: the annual figure, whether it is paid biweekly or semimonthly, whether it is subject to proration, and whether it is stated as "guaranteed" or "targeted." The word matters. A "guaranteed" base is payable regardless of production. A "targeted" base is subject to reconciliation against production.

Production compensation

Confirm: the wRVU multiplier dollar amount, the production threshold, whether the threshold is absolute or percentage-based, and whether there is a cap on production compensation. Caps are unusual in IM contracts and should be negotiated out if present.

Quality and performance bonuses

This is where many contracts obscure the structure. Confirm: the specific metrics used (HEDIS, STAR ratings, patient experience scores, readmission rates), the targets for each metric, the weight of each metric, the maximum possible payout, and whether the metrics are calculated at the physician level, the group level, or the system level. **System-level metrics are often structurally unreachable by individual physician effort.**

Call and shift differentials

For hospitalists and outpatient IM with call, confirm: the rate paid per call shift (or per call day), whether call is in addition to or included in base, whether weekend and holiday calls are paid differently, and whether nocturnist shifts carry a separate differential. Rates should be specified in dollars, not percentages, to avoid ambiguity.

Sign-on and retention bonuses

Confirm: the amount, the payment schedule (typically 50% at start, 50% at 6 or 12 months), the clawback structure (pro-rata over 24 months is the standard; 36+ months or full repayment is aggressive), and

whether clawback applies to termination without cause by the employer (it should not — this is negotiable).

CME and professional development

CME budgets of $3,000–$5,000 annually for IM are common; $5,000–$7,500 is more favorable.

Malpractice coverage

Confirm: whether coverage is claims-made or occurrence, the policy limits ($1M / $3M typical), who provides the coverage, and — critically — who pays the tail on termination if coverage is claims-made. Tail cost for IM is typically $15,000–$40,000. This is one of the most important clauses in the contract.

Reimbursement for specific expenses

Confirm: licensure, DEA, board certification, hospital privileging fees, professional society memberships. These should be employer-paid; if not specified, they will default to physician-paid.

Pre-contract bonuses

If they are offered verbally but not in the written schedule, **they do not exist.** Get every dollar figure in writing.

Chapter 8

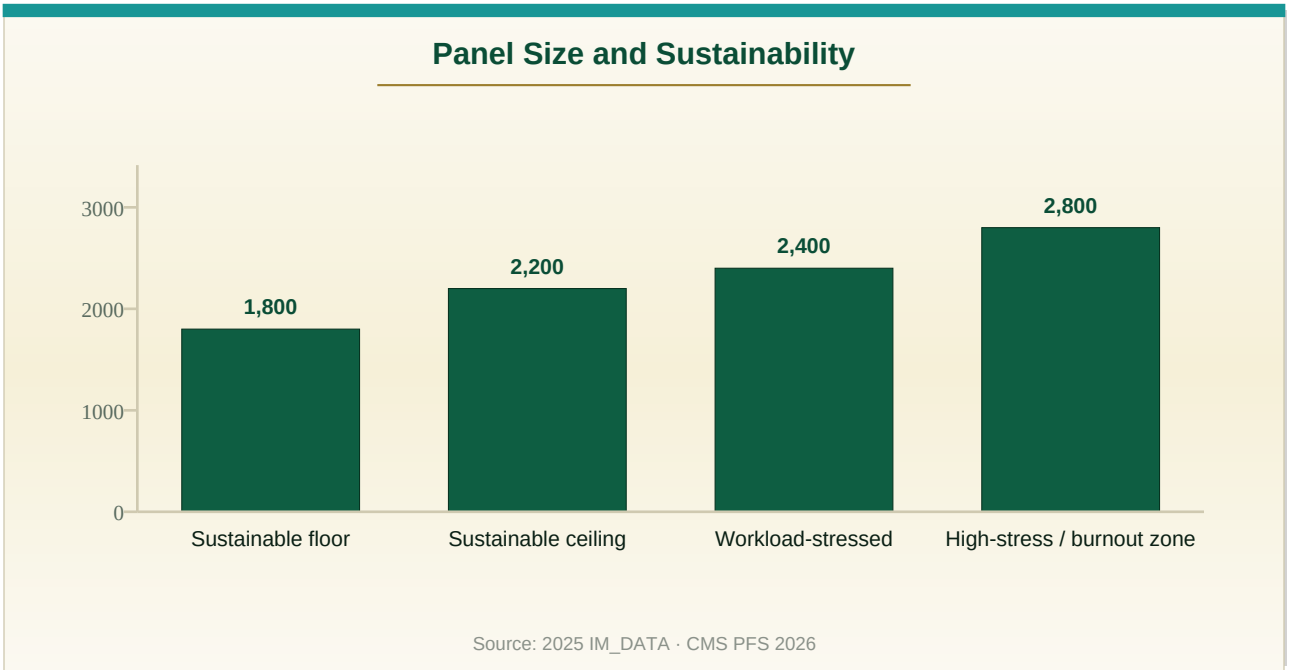
The Panel Size Trap

Panel size is the single most underappreciated compensation variable in outpatient Internal Medicine. It is a hidden ceiling on your wRVU production, a driver of unpaid administrative work, and the lever that most often separates a sustainable IM contract from an unsustainable one.

Panel size refers to the total number of patients attributed to you for primary care purposes. The attribution methodology varies — some systems use the last-four-visits rule, some use the most-recent-visit rule, some use payer-defined attribution under an ACO or MA contract. Whatever the methodology, the panel is the universe of patients for whom you are the designated PCP.

Sustainable panel range

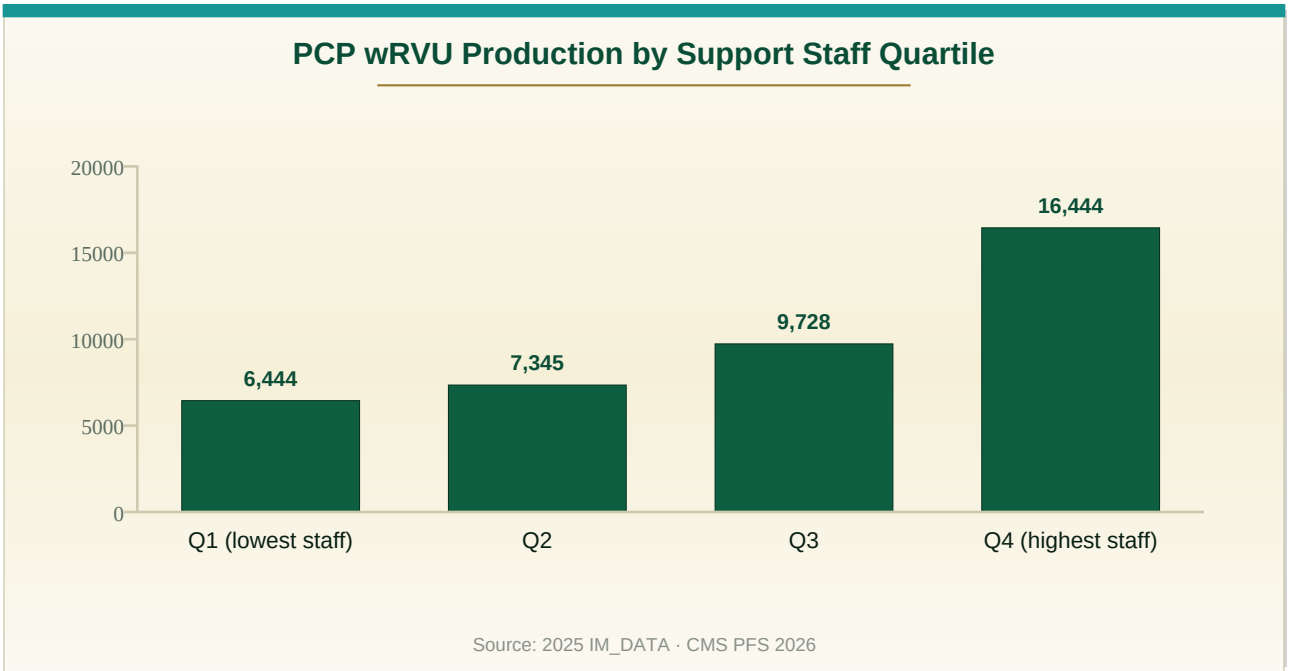
Sustainable panel sizes for full-time outpatient IM are widely cited as 1,800–2,200. Above 2,200, administrative workload — messages, refills, results review, prior authorizations, forms — grows faster than clinical workload, consuming time that is not generating wRVUs.



Support staff is the other half of the equation

Panel size is one half of the wRVU ceiling. Support staff is the other. Published 2024 PCP support-staff data shows primary care physicians supported by the highest-quartile staffing produce 2.5× the wRVUs of

physicians supported by the lowest-quartile staffing.



If your panel is growing without commensurate staff support, the wRVU ceiling falls — not the floor.

Three components of the trap

1 Many outpatient IM contracts specify neither a panel cap nor a per-patient compensation adjustment above a threshold. As the group loses physicians who are not replaced, panels grow. The remaining physicians absorb the additional patients without additional compensation.

1 Panel growth does not produce proportional wRVU growth. A physician can see only so many patients per day. If your panel grows by 33% but your appointment slots are fixed, you do not generate 33% more wRVUs. The "invisible work" — messages, refills, phone calls, patient portal activity, care coordination — consumes time but generates no wRVUs.

1 Many contracts with quality or HEDIS bonus structures measure panel-level performance. A larger panel means more patients on which to achieve HEDIS targets. A larger panel with unchanged staffing support means worse HEDIS performance, which reduces bonus payout.

The remedies

■ The remedies in contract negotiation are two, and they are mutually exclusive. Either cap the panel or price the growth.

Panel cap: writes into the contract a maximum attributed panel size — 2,200 is a reasonable cap for full-time outpatient IM.

Per-patient compensation: writes into the contract a dollar amount paid per attributed patient above a threshold. For example: $25 per attributed patient per year above a 2,000-patient baseline. At a 2,400 panel, this structure pays an additional $10,000 per year. At a 2,800 panel, an additional $20,000.

The panel size conversation is one of the most tractable in IM contract negotiation. The data is clear, the precedent is well-established, and the structural fix is simple to implement. **It is a contract term. Negotiate it.**

Chapter 9

The Quality Bonus That Isn't

Quality and performance bonuses are a standard feature of IM contracts. They are also one of the most frequent sources of compensation disappointment. The gap between the maximum stated bonus and the typical actual payout is often substantial, and the gap is usually not explained in the contract itself.

The anatomy of a typical quality bonus: a total pool of 5–15% of base salary, tied to performance against 4–8 quality metrics, with specific thresholds for each metric and weighting across metrics.

Four structural problems recur

▲ **Problem 1: The denominator is system-level.** Many quality bonuses measure performance at the practice or hospital level, not the individual physician level. An excellent individual performer with a slightly below-average group still scores below threshold.

Problem 2: The threshold is set at or above the 75th percentile of national performance. A quality metric with a threshold requiring 90th-percentile performance is not a stretch goal — it is structurally unreachable for median performers.

Problem 3: Attribution is payer-controlled. For HEDIS measures that depend on claims-based attribution, the denominator is which patients are attributed to the physician under payer methodology. A 2% shift in attribution can move the metric across a threshold.

Problem 4: The metrics are lagging indicators. HEDIS measures typically reflect care delivered 12–24 months before the bonus year.

How to read a quality bonus in your contract

1. **Identify the attribution.** Physician level? Team level? Practice level? System level? Physician-level metrics are the most responsive to physician effort. System-level metrics are the least.
2. **Identify the threshold.** Fixed numerical, percentile-of-national, or internal benchmark? Fixed thresholds and internal benchmarks are typically more achievable than percentile-of-national thresholds.
3. **Identify the weighting.** If the bonus has six metrics and one weighted metric is system-level and structurally unreachable, a physician can excel on the other five and still miss most of the bonus.
4. **Calculate the expected value.** Multiply the maximum bonus by a realistic achievement rate. If the maximum is 10% of $280,000 ($28,000) and realistic achievement is 50% ($14,000), plan on the expected value, not the maximum.

5 **Compare the expected-value bonus to the gap between your base and the published median.** If your base is $20,000 below median and your expected-value bonus is $10,000, the bonus closes half the gap — the base is still below-market even with full bonus achievement.

Common improvements achievable in negotiation: shifting at least some metrics to physician-level attribution, lowering thresholds to achievable performance levels, removing patient-experience metrics from bonus structures (these are notoriously noise-driven), and reducing the weight of system-level metrics relative to physician-level ones.

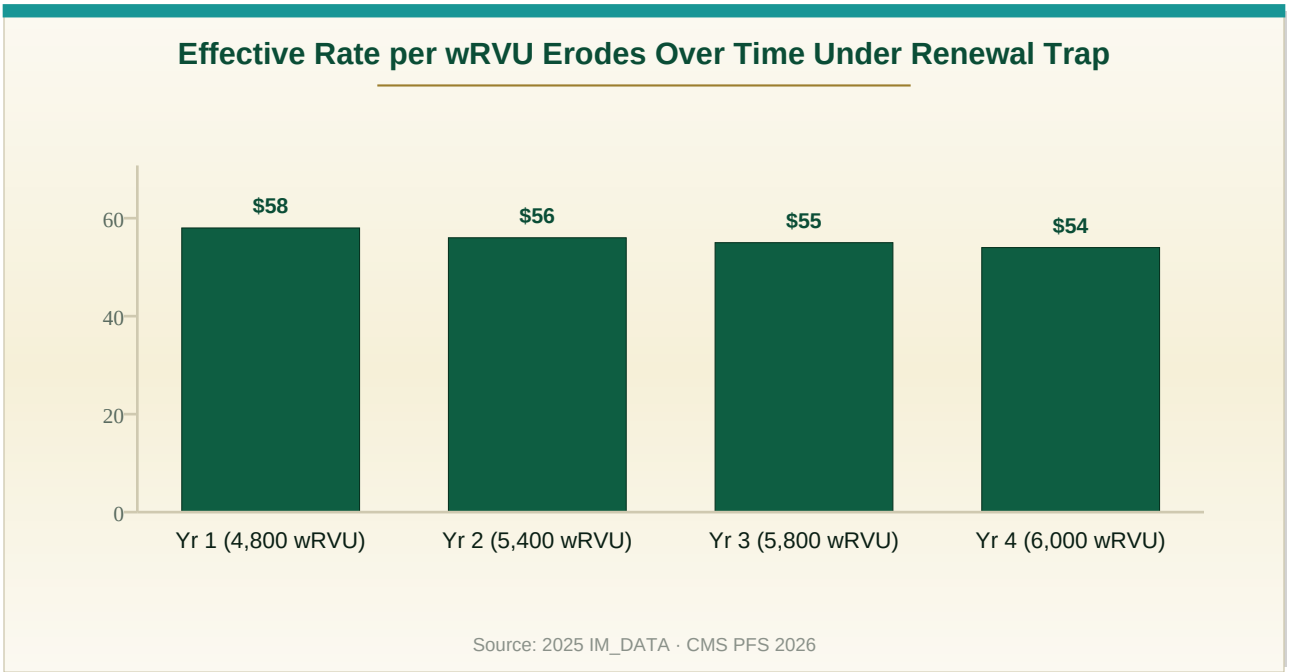
Chapter 10

The Compensation Model Switch

A specific contract pattern deserves its own chapter: the transition from salary to production at renewal, without a corresponding rate increase. This is the most common mechanism by which an initially competitive IM contract becomes uncompetitive over time.

The pattern works as follows. A first-year contract is structured as a straight salary — say, $280,000 guaranteed for Hospitalist: Internal Medicine. The employer offers this structure to give the new physician a stable income during ramp-up. The physician signs without scrutinizing the renewal mechanics. In year two or three, the contract renews, but the structure changes. The new structure is base plus production, with the production threshold set at a level calibrated to what the physician produced in year one.

Here is the trap. In year one, the physician produced 4,800 wRVUs. The renewal sets the base at $270,000 with a production threshold of 4,800 wRVUs and a multiplier of $50 per wRVU above threshold. On paper, this looks like a modest raise. But notice what happens as production grows:



The physician is producing more wRVUs each year, but the effective per-wRVU rate is going down. Working harder, paid at a lower marginal rate on the additional work. By year four, the physician is producing at the 75th percentile but being compensated at below-median per-wRVU rates.

The structural fix

★ Negotiate the renewal terms at the time of the initial contract, not at the time of renewal. Specifically, request the following language:

> *At the conclusion of the initial guaranteed-salary period, compensation shall convert to a base-plus-production structure with the production threshold set at the then-current 50th percentile of published compensation surveys' production benchmarks for the applicable specialty, and the per-wRVU multiplier set such that 100% achievement of the 50th-percentile production yields total cash compensation at the then-current 50th percentile of published total compensation. Production above threshold shall be compensated at [$X] per wRVU, with no cap.*

This language ties the renewal structure to benchmarks. It prevents the employer from setting a threshold calibrated to year-one production and forcing the physician to chase a rising target for stagnant marginal compensation.

The worst structure is the one physicians most often accept: initial contract is a three-year agreement with specific year-one terms and "to be negotiated at renewal" for subsequent years. **This hands the employer complete leverage at renewal, when the physician has sunk moving costs, built a patient panel, and has much higher friction to change employers.**

Chapter 11

Building Your Leverage File — A 90-Day Action Plan

Preparation is leverage. This is a week-by-week action plan to build a complete file before you enter any negotiation. The plan assumes you have 90 days. If you have less, compress; if you have more, deepen.

Weeks 1–2: Document collection

Pull your current employment contract. Pull your annual compensation statements for the last three years. Pull your production dashboards from your EHR. Download the actual wRVU figures for each of the last 24 months. Pull your panel attribution report for the last 12 months.

If your employer does not provide these documents on request, that is information in itself. Programs that resist transparency typically do so because transparency would reveal below-market compensation.

Weeks 3–4: Benchmark acquisition

Acquire access to current 2025 published compensation surveys for your specialty: Doximity Physician Compensation Report (free), Medscape Physician Compensation Report (free with registration), AMN Healthcare Recruiting Incentives review (free), and licensed reports such as MGMA Provider Compensation Data and SHM State of Hospital Medicine.

Week 5: Benchmark analysis

Construct your personal benchmark sheet. One page. Include current total cash compensation; published medians and percentiles for your specialty; your production (last 12 months wRVUs); your effective rate per wRVU.

Week 6: Contract review with attorney

- Hire a physician-contract attorney. Expect $500–$1,500 for a thorough review. **This is not optional.**

Week 7: Market reconnaissance

Three conversations: a recruiter call, a locum rate inquiry, and an informal conversation with peers in comparable roles.

Week 8: Script and data assembly

Write out your specific asks. Anchor each ask to data. Prepare responses to the eight most common objections (see *MedContractIntel Negotiation Script Pack*). Decide your primary ask, your acceptable counter-offer, and your walk-away. **All three must be specific numbers.**

Weeks 9–10: Rehearsal

Rehearse with a trusted colleague. Under pressure, prepared language often vanishes. Rehearsal embeds it in muscle memory.

Weeks 11–12: The conversation and the follow-up

Schedule a formal meeting, not a hallway encounter. Block 45–60 minutes. Bring your one-page benchmark sheet. Open directly with the purpose. Present data. Make the specific ask. Respond to objections with prepared language. Leave with a counter-offer or a timeline.

Within 48 hours, send a written follow-up email summarizing the conversation. Written follow-up creates a record and forces the employer to respond on the record.

Appendix A

wRVU Reference Table — 2026 Medicare Rates

Values reflect CMS CY2026 PFS Final Rule, January 2026 release. Medicare reimbursement amounts use the 2026 non-QP conversion factor of $33.40 applied to total RVU. Actual reimbursement varies by GPCI.

Outpatient established patient visits

CPT	Time	wRVU	Medicare $ (non-fac)
99212	10 min	0.70	$59
99213	20 min	1.30	$95
99214	30 min	1.92	$136
99215	40 min	2.80	$192

Outpatient new patient visits

CPT	Time	wRVU	Medicare $ (non-fac)
99202	15 min	0.93	$75
99203	30 min	1.60	$118
99204	45 min	2.60	$177
99205	60 min	3.50	$237

Initial hospital inpatient/observation care

CPT	Time	wRVU	Medicare $ (facility)
99221	40 min	1.63	$74
99222	55 min	2.60	$117
99223	75 min	3.50	$156

Subsequent hospital inpatient/observation care

CPT	Time	wRVU	Medicare $ (facility)
99231	25 min	1.00	$44
99232	35 min	1.59	$70
99233	50 min	2.40	$107

Hospital discharge management

CPT	Time	wRVU	Medicare $ (facility)
99238	≤30 min	1.50	$75
99239	>30 min	2.15	$107

Critical care services

CPT	Time	wRVU	Medicare $
99291	30–74 min	4.50	$199
99292	each addl 30 min	2.25	$100

Complexity and transitional care add-ons

Code	Description	wRVU
G2211	Longitudinal complexity add-on	0.33
99495	Transitional care, moderate	2.36
99496	Transitional care, high	3.10

Appendix B

Benchmark Reference Card

Internal Medicine — published 2025 compensation reference range

Specialty	Range	Midpoint
IM: General (outpatient)	$290K – $326K	~$308K
IM: Ambulatory Only	$290K – $325K	~$305K
Hospitalist: Internal Medicine	$283K – $370K+	~$320K
Hospitalist: Nocturnist	base + 15–25%	~base + 18%

Productivity-vs-compensation YoY (recent industry data)

- Hospitalist compensation YoY: ~+2.5%
- Hospitalist productivity (wRVU) YoY: ~+5.8%
- Hospitalist comp-per-wRVU ratio YoY: ~–1.0%
- Hospitalist median patient-facing hours/year: ~2,040
- Hospitalist median shift length: 12 hours

Hospitalist operational norms

- Typical adult hospitalist group size: ~25–30 FTE physicians
- Typical nocturnist compensation differential: 15–25%
- Typical encounters per shift: 14–18
- Admission fee (where applicable): $50–$150 per admit

Typical effective rate per wRVU

- Outpatient Internal Medicine: $50–$58 (median ~$55)
- Hospitalist: Internal Medicine: $55–$62 (median ~$58)
- Hospitalist: Nocturnist: $60–$70 (median ~$65)

Appendix C

Contract Red Flag Quick Reference

Fifteen provisions to flag when reviewing any IM or hospitalist contract. Full detail in the *MedContractIntel Internal Medicine Physician Contract Red Flag Checklist*.

1. wRVU rate with no named benchmark **(CRITICAL)**
2. Compensation model not clearly specified **(CRITICAL)**
3. Panel size with no cap or adjustment **(HIGH)**
4. Quality bonus denominator controlled by employer **(HIGH)**
5. Advance draw reconciliation without a floor **(CRITICAL)**
6. No minimum shift or census guarantee for hospitalists **(CRITICAL)**
7. Termination without cause under 90 days **(HIGH)**
8. Claims-made malpractice with no tail coverage commitment **(CRITICAL)**
9. Broad non-compete restrictions **(HIGH)**
10. Nocturnist differential below market or missing entirely **(HIGH)**
11. Ambiguous call coverage language **(HIGH)**
12. Restrictive moonlighting clause **(MODERATE)**
13. Sign-on bonus clawback with aggressive repayment terms **(MODERATE)**
14. Productivity bonus threshold set unrealistically high **(HIGH)**
15. Compensation language tied to referral volume or affiliated services **(CRITICAL)**

Appendix D

State Non-Compete Enforceability

Focus on high-density IM states. Enforceability is case-law driven in most states; specific outcomes vary. Always consult a licensed attorney in your state.

California: Physician non-competes void and unenforceable by statute (B&P Code §16600). For physicians, effectively no enforceability.

New Jersey: No statutory cap. Courts evaluate enforceability case-by-case. Typical upheld restrictions: 6 months to 2 years with reasonable geographic scope.

New York: BDO Seidman standard requires the restriction to be no greater than necessary. Typical upheld: 1 year with reasonable geographic scope.

Texas: Enforceable with limits (Tex. Bus. & Com. Code §15.50). Typical upheld: 1–2 years within a defined geographic limit.

Florida: Florida Statute §542.335 governs. Generally enforceable if reasonable. Typical upheld: 2 years with specific geographic scope.

Illinois: Case-law-driven for physicians. Typical upheld: 1–2 years.

Pennsylvania: Case-law driven. Typical upheld: 1–2 years within reasonable limits.

Ohio: Case-law driven. Typical upheld: 1–2 years.

Federal note: The FTC's 2024 rule banning most non-competes was vacated and is not currently in effect. State law governs.

Appendix E

Glossary of Contract Terms

Advance draw. A biweekly or monthly payment made against projected annual production. Year-end shortfalls may trigger clawback obligations.

APM (Alternative Payment Model). A CMS-defined category. Qualifying participants (QPs) receive the higher 2026 conversion factor ($33.5675) versus non-QPs ($33.4009).

Attribution. The methodology by which a patient is assigned to a physician's panel for primary care or for quality-metric calculation.

Base compensation. Guaranteed cash compensation, paid biweekly, independent of production.

BATNA. Best Alternative to a Negotiated Agreement. Knowing your BATNA is the foundation of negotiation leverage.

Capitation. A payment model in which the physician or group receives a fixed PMPM payment for each attributed patient.

Claims-made malpractice. A policy that covers claims reported during the policy period. Requires tail coverage purchase on termination.

Clawback. An obligation to repay previously received compensation if specific conditions occur.

CMS Conversion Factor. The dollar multiplier applied to total RVU to produce Medicare payment. 2026: $33.5675 (APM/QP), $33.4009 (non-QP).

Effective rate per wRVU. Total cash comp ÷ annual wRVU production. The single most useful benchmarking metric.

GPCI. Geographic Practice Cost Index — regional adjustment to each RVU component.

HCC. Hierarchical Condition Category — CMS risk-adjustment methodology.

HEDIS. Healthcare Effectiveness Data and Information Set — standardized quality metrics.

MIPS/MACRA. Federal framework for Medicare physician payment adjustments tied to quality, cost, and improvement activities.

MS-DRG. Medicare Severity Diagnosis Related Group — Medicare hospital inpatient facility payment system.

Occurrence malpractice. Covers incidents during the policy period regardless of when claim is reported. No tail required.

Panel size. Number of patients attributed to a PCP for primary care responsibility.

PE/MP RVU. Practice Expense / Malpractice RVU components of total RVU.

PMPM. Per Member Per Month — capitation payment.

QP. Qualifying APM Participant — eligible for the higher 2026 CF.

RAF. Risk Adjustment Factor — CMS-assigned score for patient complexity.

RBRVS. Resource-Based Relative Value Scale — underlying system for Medicare payment.

Tail coverage. Endorsement purchased on termination of a claims-made policy.

wRVU. Work RVU — the work-effort component, basis of most physician compensation contracts.

Appendix F

Recommended Resources

Compensation benchmarks (free or low-cost)

- Doximity Physician Compensation Report (doximity.com/reports)
- Medscape Physician Compensation Report (medscape.com)
- AMN Healthcare Physician Recruiting Incentives Review (amnhealthcare.com)

Compensation benchmarks (subscription)

- MGMA DataDive Provider Compensation (mgma.com)
- AMGA Medical Group Compensation and Productivity Survey (amga.org)
- SHM State of Hospital Medicine Report (hospitalmedicine.org)

Regulatory and reimbursement

- CMS Medicare Physician Fee Schedule (cms.gov/medicare/payment/fee-schedules/physician)
- CMS Quality Payment Program (qpp.cms.gov)

Physician-contract legal and advisory resources

- American College of Physicians (ACP) resources on contract negotiation
- Society of Hospital Medicine (SHM) practice management resources
- State medical society physician contract review services

BACK MATTER

Methodology & Sources

Methodology. Compensation benchmarks reflect 2025 published surveys (Doximity, Medscape, MGMA). RVU and Medicare reimbursement values reflect CMS CY2026 PFS Final Rule, January 2026 release. Data last reviewed: April 2026.

Disclaimer. This guide is for educational and informational purposes only. It does not constitute legal, financial, or employment advice. Consult a licensed attorney and/or financial advisor for guidance specific to your situation. Individual contract outcomes will vary.

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